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greenlight by Medivis

Business Model & Pricing Strategy

AI-Powered Prior Authorization for Imaging, Surgery & Procedures

6	9	$6M+	5-8x
MODELS ANALYZED	COMPETITORS	24-MO ARR TARGET	CUSTOMER ROI

Prepared: March 2026

Classification: Confidential

SECTION 01

Market Context

The prior authorization automation market is large, growing, and underserved in imaging and surgery.

Market Size & Growth

METRIC	VALUE	SOURCE
Total PA admin waste (US)	$41.4 - 55.8B / year	IDC 2024
Provider-side PA burden	$31B / year	Valer Health 2024
PA automation platform market (2024)	$2.18B	Allied Market Research
PA automation platform market (2033)	$11.32B (18.4% CAGR)	Allied Market Research
FHIR PA market (2025 → 2036)	$745M → $3.2B	Industry estimates
Total medical PA transactions / year	350 - 500M	CAQH 2024
Medicare Advantage PA determinations	53M (2024), up from 37M (2019)	KFF 2024

The Automation Gap

PA CATEGORY	REVENUE PROTECTED	AUTOMATION RATE
Standard pharmacy	$15 - $500	~75% electronic
Imaging	$500 - $4,000	~30% electronic
Surgical / procedural	$10,000 - $100,000+	~25% electronic

Key insight: Surgical PA protects 100-200x the revenue of pharmacy PA, yet has the least automation. 80.7% of appealed MA denials are overturned (KFF 2024), but only 11.5% of denials are actually appealed — representing $10B+ in lost revenue annually.

Regulatory Tailwind: CMS-0057-F

- **Jan 1, 2026 (now in effect):** 72-hour urgent / 7-day standard decision timeframes
- **Jan 1, 2027:** All MA, Medicaid, CHIP, QHP payers must support FHIR-based PA API
- Covers medical items/services — explicitly excludes pharmacy (GreenLight's exact niche)

SECTION 02

Base Assumptions

PA volumes, staff costs, and financial model inputs used across all projections.

PA Volume by Facility Type

FACILITY TYPE	AVG PAS / MONTH	ORDERING PROVIDERS
Solo Radiology Practice	80	1 - 2
Multi-Physician Imaging Center	250	5 - 8
Ambulatory Surgical Center	350	8 - 15
Community Hospital	1,500	50 - 100
Large Health System	8,000	200 - 500

Cost of Manual PA Processing

COST COMPONENT	ESTIMATE	SOURCE
Avg time per PA (phone + fax + follow-up)	45 minutes	AMA 2024
Loaded staff cost (PA coordinator)	~$55 / hour	BLS + benefits
Manual PA cost (CAQH)	$10.26 provider-side	CAQH 2022
Electronic PA cost (CAQH)	$1.92 (81% reduction)	CAQH 2022
Denial rework cost	$50 - $120 per denial	Valer Health
Annual PA burden per physician	$68,274 (2.0 FTE equiv)	AMA 2024

$42
AVG LABOR COST PER PA

$30
EFFECTIVE SAVINGS PER PA

70%
TARGET AUTOMATION RATE

15%
AVG IMAGING DENIAL RATE

SECTION 03

Competitive Landscape

The PA automation market is fragmented. No player dominates imaging and procedure PA with self-serve + AI autonomy.

COMPANY	MODEL	PRICE POINT	FOCUS	FUNDING
Humata Health	Enterprise	$50K-200K/yr	Imaging PA + EHR	$25M Series A
Infinitus AI	Per-call	$25-50/call	AI phone agent	$50M Series B
Myndshft/Waystar	Per-txn	$3-10/PA	Real-time PA	Acquired
Tandem Health	Pharma-funded	Free to providers	Medication/Rx PA only	$137M ($1B val)
Cohere Health	PMPM	$0.50-2 PMPM	Payer PA platform	$106M Series C
Waystar	Enterprise	$50K-500K/yr	Full RCM suite	Public ($4.7B)
SamaCare	SaaS + txn	$500-1.5K/mo	PA submission portal	$20M raised
CoverMyMeds	Pharma-funded	Free to providers	Pharmacy PA only	Acq. $1.4B
Availity	Payer-funded	$0-500/mo	Clearinghouse + PA	Private

GreenLight by Medivis differentiation: Purpose-built for imaging and surgical PA. Self-serve SaaS onboarding (no enterprise sales cycle required). AI-powered autonomy engine (auto-submit, auto-appeal, clinical summarization). FHIR-native with Da Vinci CRD/DTR/PAS. ACR Appropriateness Criteria matching. No competitor combines all of these with self-serve pricing.

SECTION 04

Business Model Options

Six pricing models evaluated. Each has trade-offs in revenue predictability, value alignment, and go-to-market fit.

Model A: Per-Transaction

Charge a flat fee for every PA processed through GreenLight.

Pricing: $6-12 per PA submitted (recommended: $8)

Competitors: Infinitus ($25-50/call), Myndshft ($3-10/PA), Tandem (est. per-case)

Best for: Mid-to-large facilities with predictable PA volume

Pros	Cons
+ Directly tied to value — easy ROI story	- Revenue unpredictable month-to-month
+ Scales with customer volume	- Harder to forecast ARR for fundraising
+ Low barrier to entry	- Customers may route only difficult PAs
+ Variable revenue aligns with usage	- No revenue floor during slow months

Model B: Flat SaaS Subscription

Fixed monthly fee per tier, regardless of PA volume.

Pricing: Starter $299/mo | Professional $799/mo | Enterprise custom

Competitors: Most B2B SaaS, some healthcare workflow tools

Best for: Self-serve SMB acquisition, initial ARR building

Pros	Cons
+ Predictable MRR — investors love this	- Disconnected from value at high volume
+ Simple to understand and sell	- Hospital pays same as solo practice
+ Self-serve via Stripe Checkout	- Revenue capped regardless of usage
+ Standard SaaS metrics (MRR, churn, LTV)	- Must gate features, not volume

Model C: Hybrid (Base + Per-Transaction)

Monthly base fee includes X PAs, then per-PA overage pricing.

Pricing: $799/mo (250 PAs incl.) + $6/PA overage

Competitors: Twilio, AWS, most mature health-tech platforms

Best for: Bridging self-serve and enterprise — RECOMMENDED

Pros	Cons
+ Predictable base MRR + volume upside	- Slightly more complex billing
+ Revenue scales with customer value	- Two-component pricing to explain
+ Stripe supports natively (metered billing)	- Requires usage tracking infrastructure
+ Fair pricing at every customer size	
+ Best investor story: predictable + growth	

Model D: Per-Provider Seat Licensing

Charge per ordering provider who uses the platform.

Pricing: $100-300 per provider per month

Competitors: Epic App Orchard apps, some EHR add-ons

Best for: Enterprise hospital system deals

Pros	Cons
+ Scales with practice size naturally	- Doesn't reflect PA volume
+ Easy for per-headcount hospital budgets	- Incentivizes seat minimization
+ Predictable per-customer revenue	- Misaligned with value delivery

Model E: Performance / Success-Based

Charge only when GreenLight achieves a successful outcome.

Pricing: $15-50 per approved PA — or — 1-3% of revenue protected

Competitors: Some RCM companies, contingency billing services

Best for: Pilot deals with risk-averse health systems

Pros	Cons
+ Easiest sale: 'you only pay when we deliver'	- Hard to attribute causation
+ Massive upside if approval rates are high	- Revenue timing: payment after approval
+ Eliminates buyer risk completely	- Requires trust + measurement infra
+ Powerful competitive differentiator	- Risk of low-quality PA cherry-picking

Model F: Freemium + Usage-Based AI

Free tier for manual PA tracking. Pay per AI feature usage.

Pricing: Free (manual) | $0.50-1.00/AI call | $X/mo for autonomy

Competitors: Notion, Slack, Figma (rare in healthcare)

Best for: Long-term market capture if well-funded

Pros	Cons
+ Massive top-of-funnel	- Free users cost money
+ Product-led growth: value before payment	- Healthcare SaaS conversion 2-5%
+ AI usage tied to AI costs	- Long sales cycle free → paid
+ Natural upgrade path	- Difficult to build ARR quickly

SECTION 05

Financial Model Comparison

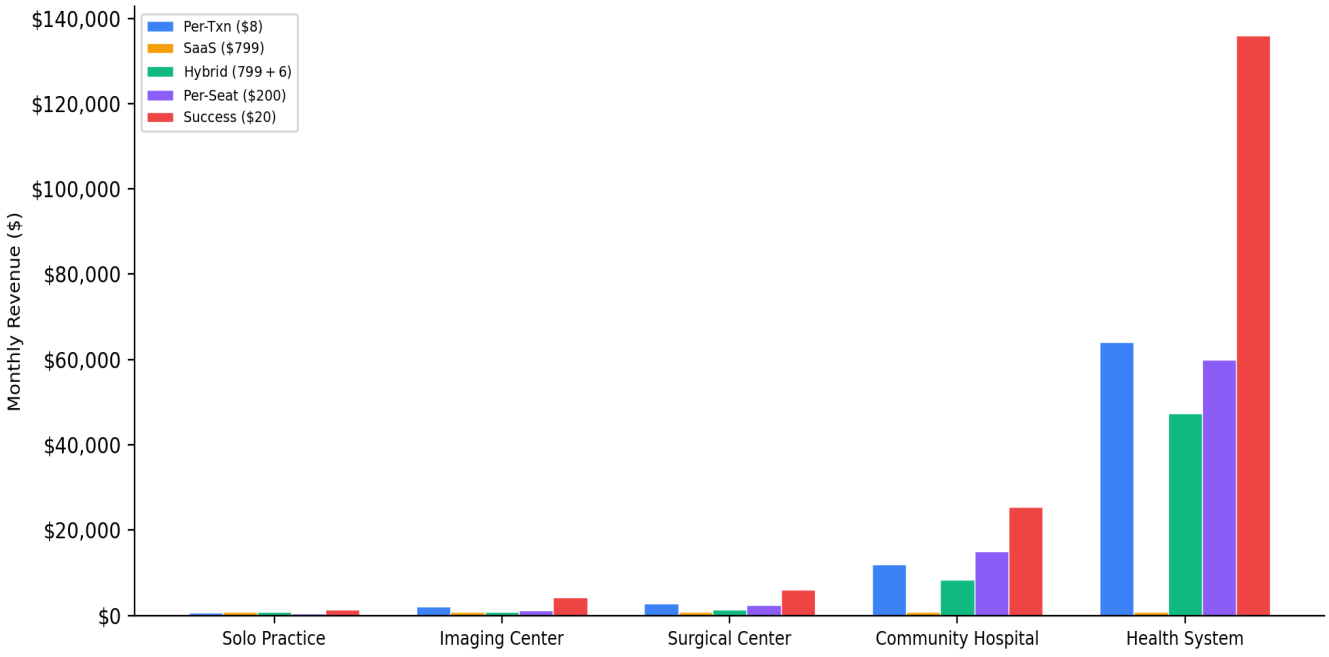
Revenue projections under each model. Base case assumptions from Section 02.

Revenue Per Customer (Monthly)

CUSTOMER	PER-TXN $8	SAAS $799	HYBRID	PER-SEAT	SUCCESS
Solo Practice (80 PAs)	$640	$799	$799	$400	$1,360
Imaging Center (250 PAs)	$2,000	$799	$799	$1,200	$4,250
Surgical Center (350 PAs)	$2,800	$799	$1,399	$2,400	$5,950
Community Hospital (1500 PAs)	$12,000	$799	$8,299	$15,000	$25,500
Health System (8000 PAs)	$64,000	$799	$47,299	$60,000	$136,000

Observation: Flat SaaS massively undercharges high-volume customers ($799 for a hospital doing 1,500 PAs/mo). Per-transaction and hybrid models capture 5-15x more revenue from hospitals and health systems. The hybrid model provides the best balance across all customer sizes.

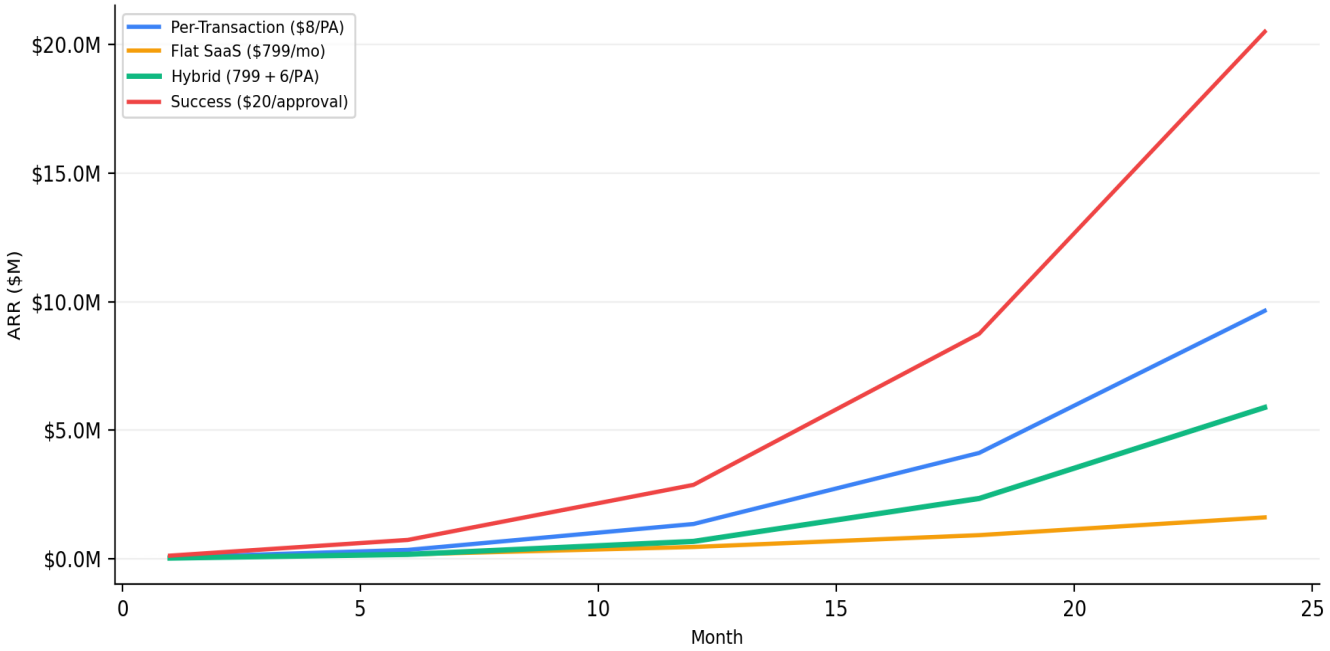
Revenue by Facility Type



SECTION 06

ARR Projection — 24 Months

Customer acquisition: 3/mo (M1-6), 5/mo (M7-12), 8/mo (M13-18), 12/mo (M19-24). 5% monthly churn.



MODEL	MONTH 12 ARR	MONTH 24 ARR
Per-Transaction ($8/PA)	$1.35M	$9.65M
Flat SaaS ($799/mo)	$0.46M	$1.61M
Hybrid ($799 + $6/PA)	$0.68M	$5.89M
Success ($20/approval)	$2.88M	$20.50M

SECTION 07

Customer ROI Analysis

Net savings vs. GreenLight cost under the hybrid model. ROI excludes denial prevention and revenue protection value.

FACILITY	PAS/MO	MONTHLY SAVINGS	GREENLIGHT COST	NET SAVINGS	ROI
Solo Practice	80	$2,400	$799	$1,601	200%
Imaging Center	250	$7,500	$799	$6,701	839%
Surgical Center	350	$10,500	$1,399	$9,101	651%
Community Hospital	1500	$45,000	$8,299	$36,701	442%
Health System	8000	$240,000	$47,299	$192,701	407%

Every customer segment sees positive ROI. Solo practices achieve 2x ROI. Hospitals and health systems see 5-8x ROI — making the sale virtually risk-free. These figures exclude additional value from denial prevention and auto-appeal revenue recovery, which can add 3-10x more.

2x+
SOLO PRACTICE ROI

5-8x
HOSPITAL ROI

$10B+
UNAPPEALED DENIAL VALUE

80.7%
APPEAL OVERTURN RATE

SECTION 08

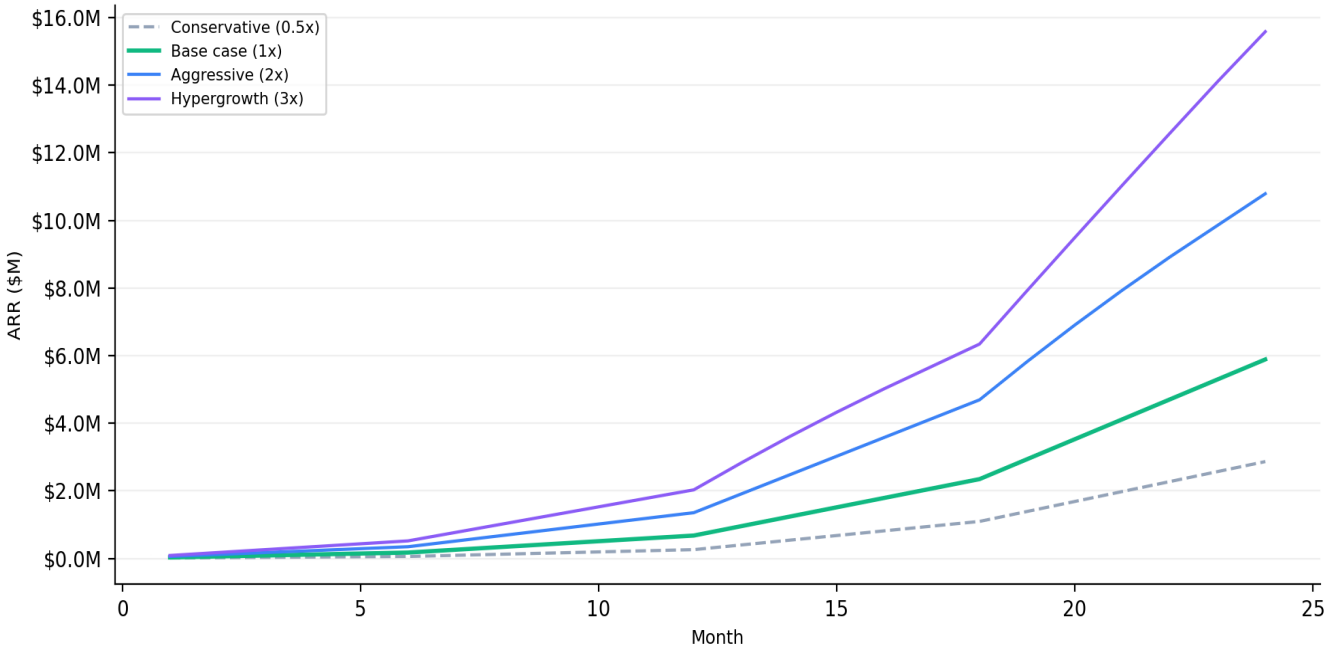
Sensitivity Analysis

Key levers that drive revenue under the hybrid model.

Price Sensitivity (Imaging Center, 250 PAs/mo)

SCENARIO	BASE/MO	INCL PAS	OVERAGE	MONTHLY	ANNUAL
Low base, low overage	$299	100	$4	$899	$10,788
Low base, high overage	$299	100	$8	$1,499	$17,988
Mid base, mid overage	$499	150	$6	$1,099	$13,188
Recommended	$799	250	$6	$799	$9,588
High base, low overage	$999	300	$5	$999	$11,988
Premium	$1499	500	$4	$1,499	$17,988

Customer Acquisition Sensitivity (Hybrid Model)



SECTION 09

Recommendation

Model C (Hybrid) as primary pricing, with Model E (Success-Based) as a pilot incentive for enterprise.

Recommended Pricing Structure

	STARTER	PROFESSIONAL	ENTERPRISE
Monthly base	$299	$799	Custom
Included PAs	50	250	Unlimited
Overage per PA	$8	$6	Negotiated
AI calls	100 included	Unlimited	Unlimited
AI overage	$0.50/call	—	—
EHR integration	—	Included	Dedicated
Autonomy engine	—	Included	Included
Trial	14 days	14 days	30 days
Target	Solo / small practice	Imaging / surgical center	Hospital / health system

Go-to-Market Phasing

- **Phase 1 (Now – Q3 2026):** Launch hybrid pricing. Self-serve signup for Starter and Professional. Target 10-20 imaging centers.
- **Phase 2 (Q4 2026):** Add success-based pilot for enterprise leads. '$0 for 90 days' to de-risk hospital deals.
- **Phase 3 (Q1 2027):** CMS-0057-F FHIR PA mandate. Position as turnkey compliance solution. Regulatory urgency marketing.
- **Phase 4 (2027+):** Per-transaction for high-volume systems. Revenue-share for auto-appeal (% of recovered revenue).

Key Metrics to Track

METRIC	MONTH 6	MONTH 12	MONTH 24
Customers	15-20	40-50	120-150
MRR	$15K-25K	$60K-100K	$300K-500K
ARR	$180K-300K	$720K-1.2M	$3.6M-6M

METRIC	MONTH 6	MONTH 12	MONTH 24
Avg Revenue / Customer	$1,200/mo	$1,800/mo	$3,000/mo
Net Revenue Retention	>110%	>120%	>130%
Monthly Churn	<5%	<4%	<3%

The hybrid model wins because it serves the self-serve long tail (imaging centers) AND captures proportional revenue from enterprise (hospitals). It provides predictable base MRR for investors while scaling with customer success. Stripe supports it natively. No competitor offers this with AI-powered autonomy and self-serve onboarding.